

ORTHOPEDIC CONSULTATION NOTE

Patient: Sam Rodriguez | DOB: 1972-09-04 | Sex: M | MRN: EHR-SR-001
Date: 2026-05-05 | Provider: Dr. James Patel, MD | Specialty: Orthopedic Surgery
Place of Service: Office (POS 11)

CHIEF COMPLAINT

Right knee pain for 4 months, worsening, not responding to treatment.

HISTORY OF PRESENT ILLNESS

Mr. Sam Rodriguez is a 53-year-old male presenting with a 4-month history of right knee pain. He describes the pain as aching and sharp with certain movements, particularly with twisting, squatting, and ascending stairs. He reports a popping sensation in the right knee that began approximately 4 months ago during a recreational soccer game. Since that time, he has had persistent pain along the medial joint line, intermittent swelling, and occasional locking/clicking sensations with knee flexion.

Pain severity: 6/10 at baseline, up to 9/10 with provocative activities. Worse with pivoting and prolonged walking. The patient reports that conservative measures have been tried per his primary care physician's recommendation; however, despite these efforts, he has not experienced meaningful improvement in pain or function, and his symptoms have progressively worsened. He has been unable to return to his usual recreational activities.

Note: The patient verbally reports that conservative measures failed per patient report. Formal physical therapy records were not available for review at today's visit. The patient reports participating in approximately 6-8 weeks of home exercise program and NSAID therapy as directed by his PCP.

No history of prior knee surgery. No prior knee injections. No locking requiring manipulation. BMI 27.

PAST MEDICAL HISTORY

- Mild hypertension (on low-dose HCTZ)
- No prior musculoskeletal surgeries
- No bleeding disorders

MEDICATIONS

- Hydrochlorothiazide 12.5 mg daily
- Ibuprofen 600 mg TID PRN knee pain (per PCP instructions)
- Naproxen 500 mg BID PRN (previously prescribed, patient reports limited benefit)

ALLERGIES

- NKDA

SOCIAL HISTORY

- Married, works in IT
- Recreational soccer player (unable to play for 4 months due to knee)
- Non-smoker, social alcohol

FAMILY HISTORY

- Mother: osteoarthritis
- Father: diabetes type 2

REVIEW OF SYSTEMS

- Positive: right knee pain, swelling, mechanical clicking/locking sensation, functional limitation
- Negative: fever, constitutional symptoms, bilateral joint complaints, back pain radiating to leg, hip pain

PHYSICAL EXAMINATION

Vital Signs: BP 130/82 mmHg, HR 74 bpm, Weight 185 lbs, Height 5'10"

General: Alert, well-appearing male. Mild antalgic gait favoring the right lower extremity.

Right Knee Examination:

- Inspection: Mild effusion present. No ecchymosis or erythema.
- Palpation: Tenderness to palpation along the medial joint line. Medial collateral ligament area mildly tender. No lateral joint line tenderness.
- Range of Motion: Flexion 0-125 degrees (limited by pain at end range). Extension 0 degrees.
- McMurray Test: Positive - pain and click with valgus stress and external tibial rotation (suggestive of medial meniscal pathology).
- Apley's Compression Test: Positive with internal rotation.
- Lachman Test: Negative.
- Anterior/Posterior Drawer: Negative.
- Valgus/Varus Stress Test: Negative at 0 and 30 degrees.
- Patellar Grind: Negative.
- Ballottement: Mild fluid wave present.

Left Knee: Unremarkable.

Neurovascular: Distal pulses intact. Sensation intact. Motor 5/5 in bilateral lower

extremities.

ASSESSMENT

Suspected medial meniscal tear, right knee. Physical exam findings are consistent with internal derangement of the right knee · positive McMurray test, medial joint line tenderness, and mild effusion · in a patient with a mechanism consistent with meniscal injury (twisting injury during athletic activity). Conservative measures have failed per patient report, with ongoing functional limitation after 4 months of symptoms.

Diagnosis: Internal derangement, right knee (M23.231 · Derangement of anterior horn of medial meniscus due to old tear or injury, right knee · working diagnosis pending MRI confirmation).

PLAN

1. Order MRI right knee without contrast (CPT 73721) · for further evaluation of suspected medial meniscal tear and to rule out associated ligamentous or chondral pathology. Imaging will directly change clinical management: if MRI confirms meniscal tear, patient will be a candidate for arthroscopic meniscal repair vs. partial meniscectomy; if negative, conservative management will be intensified.
2. Continue current anti-inflammatory regimen (naproxen 500 mg BID) while awaiting MRI results.
3. Refer for formal physical therapy evaluation in parallel (pending imaging).
4. Discuss surgical options with patient once MRI results reviewed.

ICD-10 codes: M23.231 (Derangement of anterior horn of medial meniscus due to old tear or injury, right knee) · primary working diagnosis

Signed: Dr. James Patel, MD | Orthopedic Surgery | NPI: 0987654321

Date/Time: 2026-05-05 10:45